



BESMART Provider Education

May 28, 2026

Agenda

- Buprenorphine Pharmacy Updates
- BESMART Program Description Updates
- BESMART Coding Updates
- Quality Review Tool Updates
- BESMART Program Data
- Q&A

Buprenorphine Pharmacy Updates

Buprenorphine Dose and PA Process Changes

Effective today:

For BESMART MD / DO providers

- 1) **Maximum Daily Dose (MDD) limit** for preferred buprenorphine / naloxone products is **32 mg**
- 2) **No PA required** for preferred buprenorphine /naloxone products **≤ 32 mg**
- 3) **Approved PA required** for non-preferred products up to 32mg

For Non-BESMART MD / DO providers

- 1) Quantity limit ≤ 16 mg for preferred buprenorphine / naloxone products without PA
- 2) PA required for >16 mg and up to 32 mg
- 3) If no approved PA >16 mg, it is expected the patient will taper to 8mg after 6 months

Note: No changes for mid-level prescribers

Buprenorphine Process Updates for BESMART Providers

	Dose Limits	PA Requirements	Monoproducts
MD / DO	Up to 24 mg for specific populations 32 mg/day (evidenced based dosing)	- PA for non-preferred, preferred products (which includes monoproduct) PA required for non-preferred products (which includes monoproduct, Requests outside of evidenced-based dose)	- PA required for up to 24mg PA required for up to 32 mg (Evidence based dosing)
NP / PA*	16 mg	PA required for non-preferred products (up to 16 mg)	NP/PA may prescribe in CMHC and FQHCs (Injectable permitted in other settings)

Table Key:
 Current state is in black
Updates are in red

*No changes for mid-level prescribers, in compliance with state law. All mid-level prescribers must be BESMART

BESMART Program Description Updates

BESMART Program Description (PD) Updates

The BESMART PD has been updated to clarify expectations and requirements and to provide greater flexibility in the delivery of psychosocial services.

Updates:

1. Phases of Treatment:

- Separation of Induction and Stabilization phases
- Clearer requirements and expectations for each phase

2. Distinction of Counseling and Psychotherapy

- Counseling and psychotherapy have been distinguished to support clinically appropriate, patient-centered care

3. Additional Refinements

- Additional clarifications and updates have been incorporated to enhance consistency and support program implementation

BESMART PD Updates: Phases of Treatment

1. All phases of treatment sections have been updated to include goals, requirements, expected duration and activities, and when to transition to next phase
2. Induction and stabilization phases are now separate with distinct billing codes
3. Distinction of counseling (induction phase) and psychotherapy (any phase)

BESMART PD Updates: Counseling and Psychotherapy

Scopes of Service

Term	Scope of Service
Counseling	• Address present-day issues such as stress, grief, relationship problems, or career challenges. • Short-term and transitional, supports program engagement across recovery phases. • Goal-oriented, focuses on coping skills and problem-solving. • Less intensive and not focused on deep psychological exploration.
Psychotherapy	• Addresses deeper psychological issues rooted in past experiences or unconscious processes. • Long-term (months to years) and insight oriented, using established therapeutic frameworks (e.g., psychodynamic, cognitive behavioral therapy). • More intensive; explores personality, emotional patterns, and chronic mental health conditions. • Requires advanced training and supervised clinical experience. • Used to diagnose and treat complex mental health disorders.

BESMART PD Updates: Counseling and Psychotherapy (Cont'd)

Qualified Providers

Term	Provider Types
Counselors	Licensed/ certified clinicians operating within their scope of practice, including: • Psychologist, Psychiatrist, Physician, Physician Assistant, Registered Nurse, Psychiatric-Mental Health Nurse Practitioner (PMHNP-BC); or • Licensed Professional Counselors (LPC), Licensed Clinical Social Worker (LCSW), Licensed Marriage and Family Therapist (LMFT), Licensed Alcohol and Drug Abuse Counselor (LADAC II); or • Licensed Alcohol and Drug Abuse Counselor (LADAC I); or • Certified Peer Recovery Specialist (CPRS); or • Holds at least a master's degree in counseling, psychology, or a related field and is under clinical <u>supervision</u>.* • Qualified graduate student, consistent with policy PRO 22-001 (Rev. 2)¹ *If not independently licensed, <u>must</u> be under authorized clinical supervision per scope of practice requirements.
Psychotherapists	Licensed clinicians operating within their scope of practice, including: • Psychologists (PhD or PsyD), psychiatrists (MD); or • Clinical social workers (LCSW), Licensed Marriage and Family Therapist (LMFT), Licensed Professional Counselors (LPC/MHSP), Psychiatric-Mental Health Nurse Practitioner (PMHNP-BC) • Holds at least a master's degree in counseling, psychology, or a related field and is under clinical <u>supervision</u>.* • Qualified graduate student, consistent with policy PRO 22-001 (Rev. 2)² *If not independently licensed, must be under authorized clinical supervision per scope of practice requirements.

As of today:

- Counseling may be provided by qualified providers during the **induction phase**
- Psychotherapy may be provided by qualified providers during any phase

BESMART PD Updates: Additional Refinements

Additional updates were made to strengthen clarity, improve usability, and ensure the program description aligns with current standards and terminology.

- Content updated to **meet accessibility standards** and improve usability
- **Language modernized** to ensure clarity and remove stigmatizing terminology
- **Glossary added** to define key terms and support consistent interpretation
- Memorialized **psychotherapy within 7-days of E&M visit**
- Clarified that **Annual Reviews** may occur **virtual or on-site**

BESMART Coding Updates

BESMART Coding Updates

Phase of Treatment	BlueCare	UnitedHealthcare	Wellpoint
Induction	H0014 HG	H0014 HG	H0014 HG
Stabilization	G2172 HG	H0033 HG U1	H0033 HG U1
Stabilization without psychotherapy	H0033 HG	H0033 HG	H0033 HG
Maintenance	H0016 HG U1	H0016 HG U1	H0016 HG U1
Maintenance without psychotherapy	H2036 HG	H0016 HG	H0016 HG

- New billing codes take effect on August 1, 2026
- Codes will be available on [TennCare's Opioid Strategy webpage](#)

Coding Updates: More Information

- This change is regulatory - each MCO is preparing notifications and updated materials, including a crosswalk of old codes to new codes and the corresponding reimbursement (rates are not changing)
- Appropriate use of the identified modifiers on the claim will be key in ensuring the correct reimbursement for the codes
- Prescribers and practice management/billing staff will need to review this information as well as the program description changes to ensure alignment with expectations and program requirements

New Tool for BESMART Onboarding

While each MCO's process is a little different, there is considerable alignment on the data elements that are being collected for participation in BESMART. To help practices avoid common errors and improve the consistency and accuracy of the information being submitted, a new "cheat sheet" is now available.



The cheat sheet is not a replacement for MCO forms - it was created to assist practices and prescribers in understanding program requirements and the data needed during the BESMART screening and onboarding process.

Accurate information from the very beginning helps expedite the process!

New Tool for BESMART Onboarding (Cont'd)

Excel format:

- Identifies required fields for all prescribers and which fields apply based on status as MD/DO or mid-level
- Lists drop-down responses you may see in the form itself
- Provides help text, including links to federal and State governance resources
- Practice can pre-populate practice level information (EIN, service locations, etc.) and save as a template
- Each prescriber can populate their unique identifiers and modalities/populations served (LAIs, pregnant/post-partum, adolescents, etc.)
- Information needed is in one place and can be on hand (and often copied/pasted) when MCO forms are being completed

Quality Review Tool (QRT) Updates

QRT Updates: Appointment Frequency

Section III: Appointment Frequency	
	Induction Phase <i>Skip Standards 21-23 ONLY IF documentation of the induction phase is not available</i>
21	A patient in the induction phase of treatment has had: 1. One (1) to three (3) provider appointments scheduled weekly;
22	2. Received counseling at each induction visit. Received other supportive services (as appropriate) at each induction visit, such as; peer support, illness management and recovery, or other health and behavioral management activities;
23	3. Had one (1) observed random drug screen at least weekly.
	Stabilization Phase <i>Skip Standards 24-26 if patient is NOT in the stabilization phase.</i>
24	A patient in the stabilization phase of treatment has had: 1. Had a scheduled provider appointment at least every one (1) to three (3) weeks;
25	2. Received psychotherapy at least monthly;
26	3. Had one (1) observed random drug screen at least monthly.

- Induction and Stabilization standards have been separated
- Stabilization phase standard now reflects requirement for psychotherapy
- Phase of treatment must be clearly documented

Note: Changes are effective July 1, 2026

QRT Updates: Service Delivery

Section IV: Service Delivery	
33	There is evidence in the clinical record of patient receiving training and education on the following topics (Note: These must be documented once with initial establishment of care, and annually): (a) Treatment options, including detoxification, benefits/risks associated with each option;
34	(b) Risk of neonatal abstinence syndrome for all female patients of childbearing age (ages 15-44);
35	(c) Prevention and treatment of chronic viral illnesses, such as HIV and hepatitis C;
36	(d) Therapeutic benefits and adverse effects of treatment medication;
37	(e) Risks for overdose, and
38	(f) Overdose prevention and reversal agents.
39	There is evidence that the Controlled Substance Monitoring Database (CSMD) was queried each time and prior to a prescription being ordered (e.g. e-scribed/called in/written).
40	A psychosocial assessment was completed within 30-days of treatment initiation, by a qualified professional within the patient's treatment team.

- Training and Education is now required to be completed annually
- A psychosocial assessment is required to be completed within 30 days of treatment initiation

Note: Changes are effective July 1, 2026

BESMART Program Data

BESMART Program Data: 2025 Overview

Total BESMART Providers

662

BESMART Providers Who
Prescribed Buprenorphine

481

Total members Seen by a
BESMART Provider

14,669

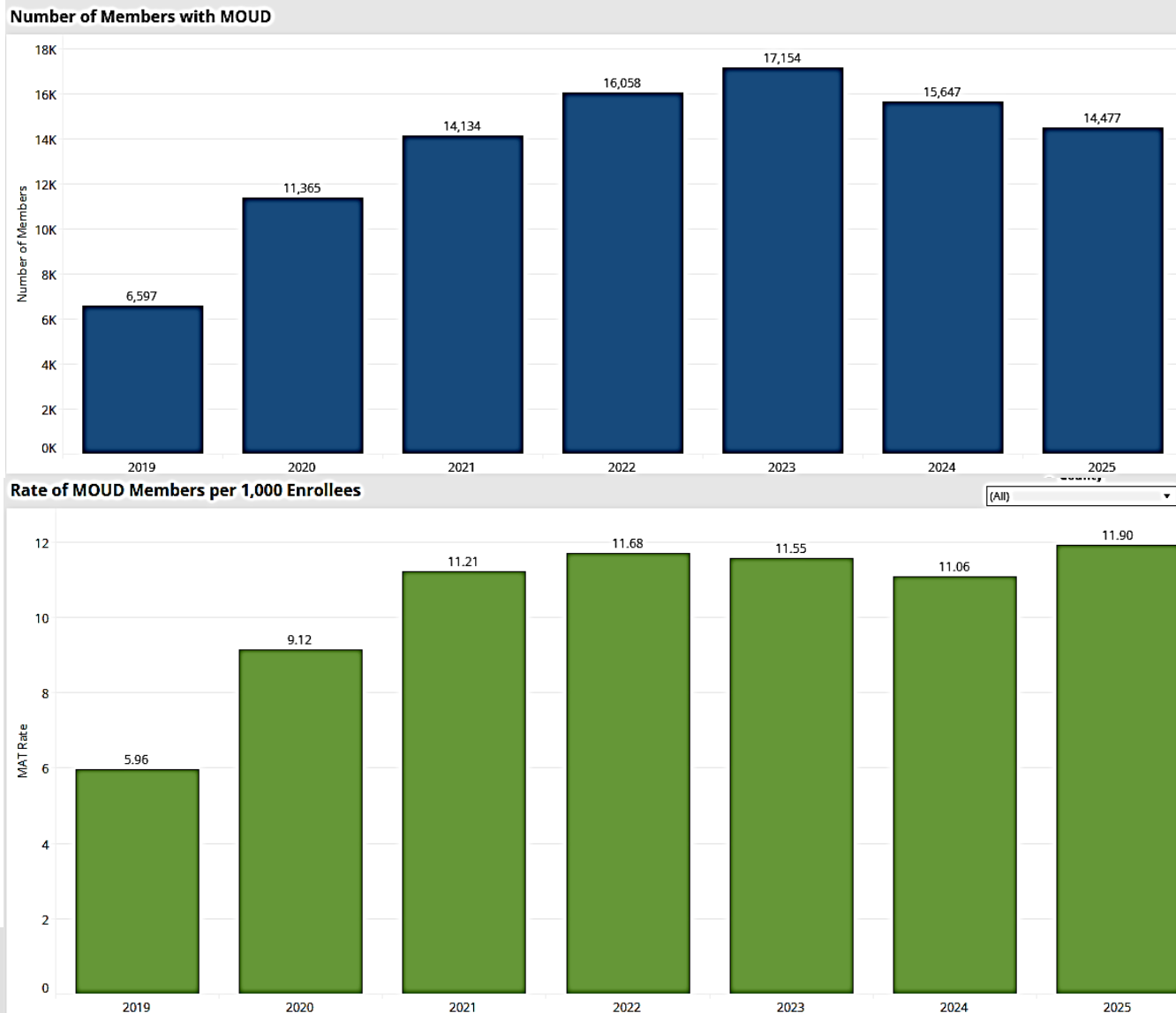
Total Non-BESMART
Providers who Prescribed
Buprenorphine

212

Total members seen
by a Non-BESMART
Provider

800

BESMART Members Receiving MOUD: Yearly Trends



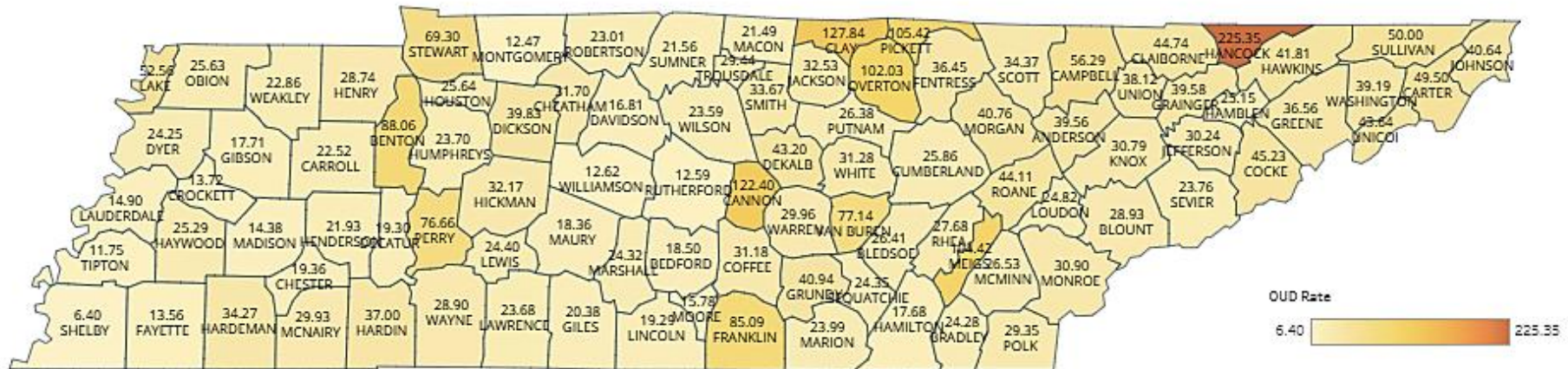
BESMART Members Receiving MOUD: 2025

Statewide Trends

Rate of OUD Members per 1,000 Enrollees

Year

2025



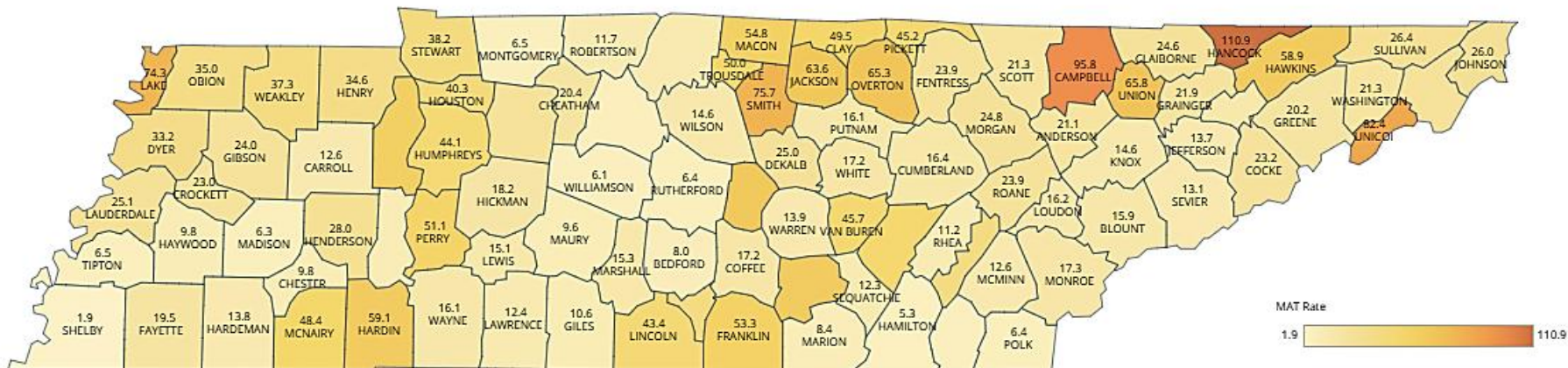
OUD Rate

6.40 225.35

Year

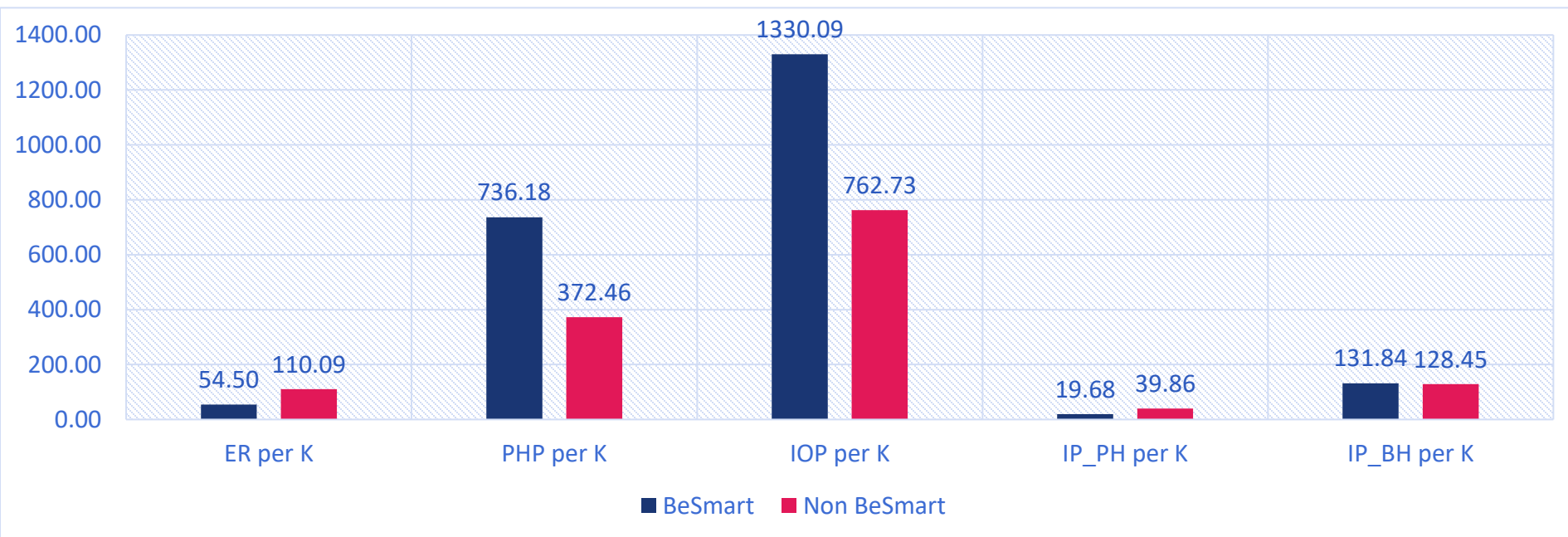
2025

Rate of MOUD Members per 1,000 Enrollees



Wellpoint Data

BESMART vs Non-BESMART



- Data based off CY2023-CY2025
- Wellpoint members engaged with BESMART = 67.55% fewer ED visits per K
- Wellpoint members engaged with BESMART 2.60% more IP BH Admits, but 67.79% fewer IP PH Admits when engaged
- Outpatient services have steadily increased when members are connected to BESMART providers



BlueCare Tennessee Data

BESMART Outcomes: Propensity-Matched Comparison

Compared to similar members who did not participate, BESMART participants experienced:

Measure	Reduction
Emergency Department (ED) Visits	12%
Inpatient Admissions (All Causes)	149%
Behavioral Health Inpatient Admissions	105%
Detox Admissions (4.0)	178%
Residential Treatment Center (3.7/3.5)	167%

Safety Signal

Mortality Rate

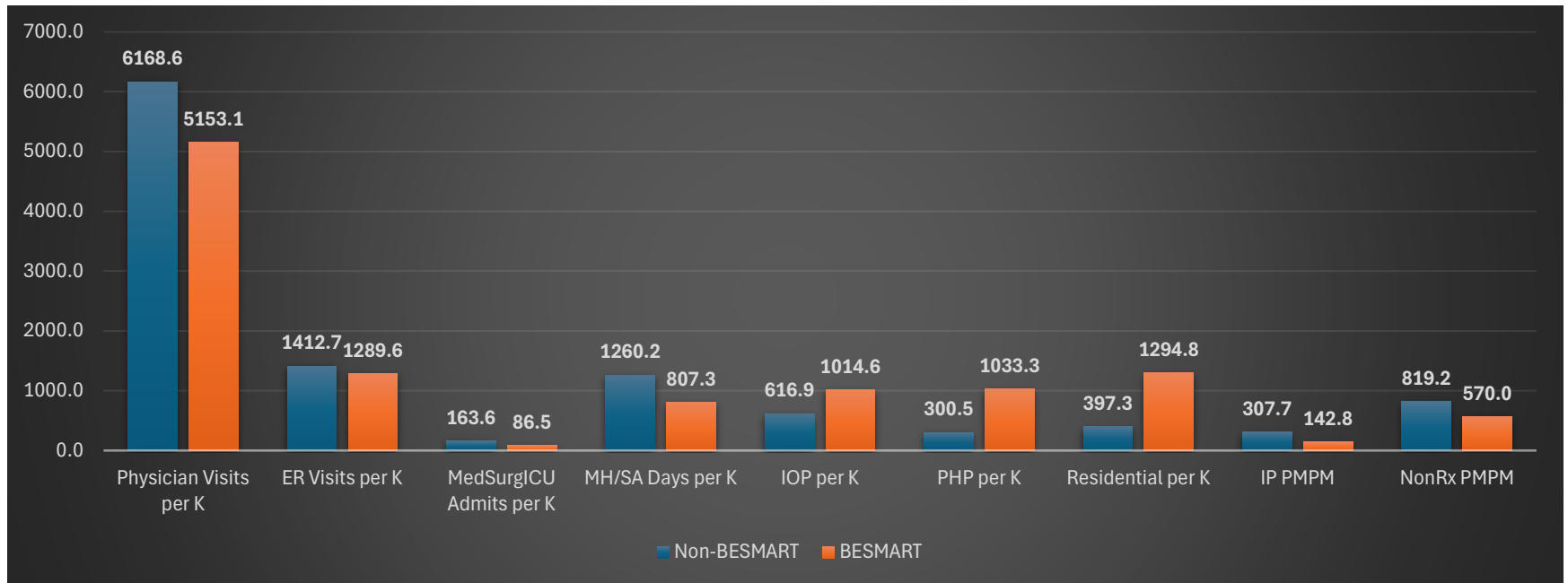
BESMART Participants: 2%

Matched Comparison Group: 8%

BESMART is associated with improved stability and safety for members

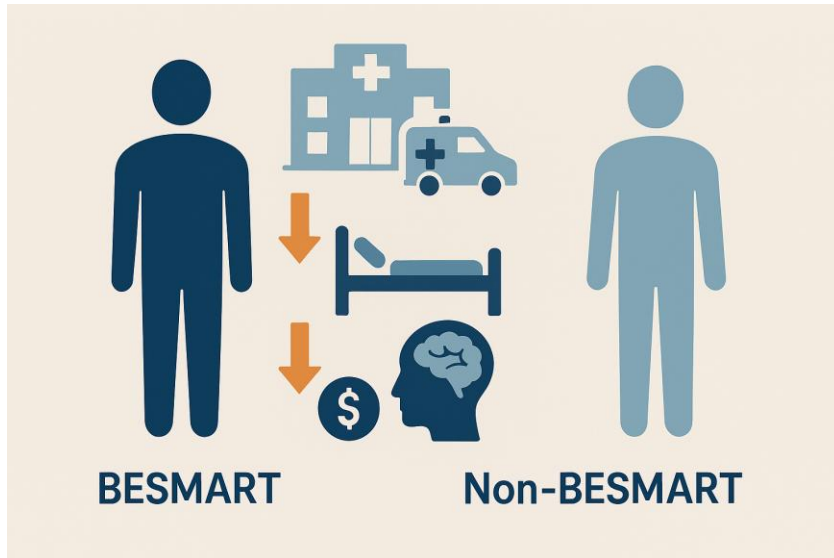
UHC Data

BESMART vs. Non-BESMART



UHC Data

BESMART vs. Non-BESMART



- Based on three-year averages spanning October 2022 through September 2025
- Emergency Department Visits 8.7% fewer for BESMART
- Med/Surg/ICU Admits 47.2% fewer for BESMART
- MH/SA Admission Days 37% fewer days for BESMART
- IP PMPM 54.4% less spend for BESMART
- Non-Rx PMPM 32.6% less spend for BESMART

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Observations and Comments

- 32% of households with a member diagnosed with SUD have a child/adolescent without a current SUD diagnosis (this is 7% higher than the national average)
- Co-occurring disorders are the most common across the total population
- MOUD presents as with lower utilization in the east and southeast in comparison to the other grand regions per SUD/OD prevalence data
- Kratom – this substance has yet to really be categorized or quantified, but providers report increasing presentation of over utilization and withdrawal from this product
- Cychlorphine (N-Propionitrile chlorphine) is a highly potent synthetic opioid, approximately 10 times stronger than fentanyl, posing extreme overdose risks



Q&A

